

Sexual Disorders

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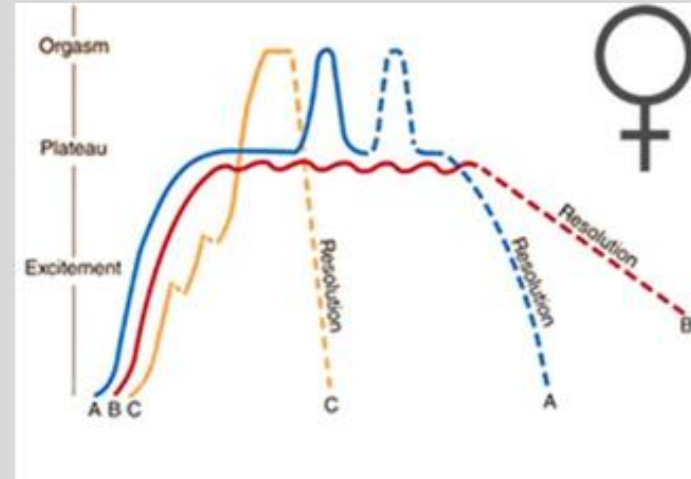
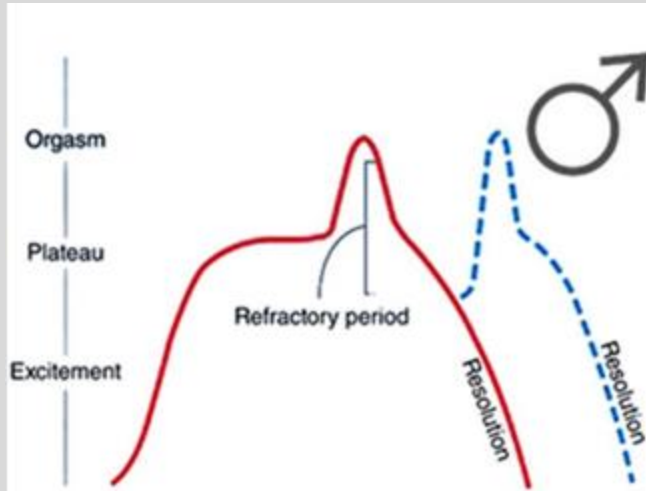
Session Objectives

- Define the general terms of sexual dysfunction and paraphilic disorders.
- Distinguish specific sexual dysfunctions.
- Apply diagnostic criteria to identify sexual dysfunctions.
- Develop a clinical management plan for sexual dysfunctions and paraphilic disorders.
- Recognize specific paraphilic disorders.

Sexual Response Cycle

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Masters, W.H.; Johnson, V.E. (1966). Human Sexual Response. Toronto; New York: Bantam Books.
[Scholar Rx Brick: Sexual Dysfunctions](#)

Definition

- Disturbance

- Sexual functioning →

Sexual Dysfunctions

- Sexual behavior →

Paraphilias

- Biological

- Psychological

- Descriptive

Sexual Dysfunction Disorders

Causes

- Medical
 - Fatigue, diabetes, hypertension, cancer, medicines, substances
- Physical
 - Prostate hyperplasia, atherosclerosis, vaginal atrophy, obesity
- Sociocultural
 - Work/Home pressures
 - Relationship dynamics
 - Remarkable stressors
 - Religion factors
 - Unrealistic expectations
- ****Psychological****
 - **Primary** vs Secondary (depression, anxiety, schizophrenia, treatments)

Symptoms and Signs

Patient Assigned Female at Birth

- Decrease vaginal lubrication
- Not able to relax pelvic musculature especially prior to or during intercourse
- Pain and/or distress during sexual intercourse
- Not able to achieve orgasm

Patient Assigned Male at Birth

- Not able to achieve or maintain an erection
- Unplanned delay or absence of ejaculation
- Premature ejaculation

Primary Sexual Dysfunctions

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- History and physical exam
 - Including sexual history
- No causes from comorbidities, stressors, relationships, or expectations
- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**

Sexual Dysfunctions

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Patient Assigned Female at Birth

- Female Sexual Interest/Arousal Disorder (FSIAD)
 - -Hypoactive Sexual Desires Disorder (HSDD)
 - -Female Sexual Arousal Disorder (FSAD)
- Female Orgasm Disorder
- Female Genito-Pelvic Pain / Penetration Disorder
 - -Vaginismus
 - -Dyspareunia

Patient Assigned Male at Birth

- Male Hypoactive Sexual Desire Disorder
- Male Erectile Disorder
- Delayed Ejaculation
- Premature (Early) Ejaculation
- Priapism

Arousal Disorders

Reduction or absence of at least 3:

- In a female or male, absent/reduced:
 - Sexual **interest/activity**
 - Erotic **thoughts/fantasies**
 - **Initiation/unreceptive** to sexual activity
 - Sexual **excitement or pleasure**
 - Response of arousal to **internal or external stimuli**
 - **Genital or non-genital sensations**
- **Almost all occasions** of sexual activity
- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**

Arousal Disorders

- Female Sexual Interest - Arousal Disorder
- Male Hypoactive Sexual Desire Disorder
- Differential Diagnosis (female or male)
 - Chronic medical condition
 - Substance use
 - Alcohol, marijuana, cocaine
 - Medications
 - Hypertension, psychotropic
 - Psychiatric/Psychological
 - Anxiety, Depression
 - (Peri-)Menopause
 - Radiation and chemotherapy
 - Lesion of the innervation of the genitalia
 - Unknown

Balon R, Segraves R (2019). Chapter 24. Sexual Dysfunction and Paraphilias. In Ebert M.H., Leckman J.F., Petrakis I.L. (Eds), *CURRENT Diagnosis & Treatment: Psychiatry*, 3e.

[Scholar Rx Brick: Sexual Dysfunctions](#)

Female Sexual Interest - Arousal Disorder

- Epidemiology
 - Prevalence – unknown
 - European study:
 - ~10-15% age < 50
 - ~25-35% age > 50
- Testing
 - Female Sexual Function Index
- Treatment
 - Treat any identified underlying cause
 - Lifestyle modifications
 - Bupropion
 - Topical lubricants/moisturizers
 - Estrogen
 - Testosterone
 - Cognitive Behavioral Therapy/Sex therapy

Male Hypoactive Sexual Desire Disorder

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- Epidemiology
 - Prevalence – 1.8% (ages 16-44 years)
- Testing
 - Psychological testing not required for diagnosis
- Treatment
 - Testosterone
 - Cognitive Behavioral Therapy (CBT)
- Prognosis
 - Transient – good
 - Chronic – poor

Female Orgasmic Disorder

In a female, persistent or recurrent:

- **Delay in, infrequency of, or absence of orgasm** following normal sexual excitement phase

AND/OR

- Marked **reduced intensity** of orgasmic sensations

WITH

- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**

Female Orgasmic Disorder

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- **Epidemiology**
 - British sample – 3.7%
- **Differential Diagnosis**
 - Psychiatric
 - Anxiety, Depression
 - Drug induced
 - Antidepressants, sedatives
 - Chronic medical condition
 - Neurologic or endocrine disorders
 - Substance use
 - Alcohol, marijuana, cocaine
 - Relationship stressors
- **Clinical Evaluation**
 - Sexual history
 - Medication history
 - Physical exam
- **Testing**
 - Female Sexual Function Index
- **Treatment**
 - Change medication
 - Cognitive Behavioral Therapy/counseling
- **Prognosis good**
 - Long term committed relationships

Genito-Pelvic Pain / Penetration Disorder

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Recurrence of pain (at least one of following) during vaginal penetration or intercourse:

- Difficulty having intercourse
- Genito-/vulvovaginal or pelvic pain
- Fear or anxiety of genital pain or vaginal penetration
- Tensing or tightening of the pelvic floor muscles with attempted penetration

- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**
- **Possible history of sexual/physical/mental abuse** resulting in increased response to perineal contact

- Dyspareunia → Genito-Pelvic Pain
- Vaginismus/vulvodynia → Penetration Disorder

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Genito-Pelvic Pain / Penetration Disorder

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- Epidemiology
 - North America – 15% c/o recurrent pain during intercourse
- Differential Diagnosis
 - Chronic medical illness
 - Endometriosis, constipation, irritable bowel syndrome, pelvic inflammatory disease, vaginal atrophy, lichen sclerosis or other perineal dermatologic disease
 - Psychiatric
 - Anxiety, post traumatic stress disorder, psychosomatic vs somatic disorder

Genito-Pelvic Pain / Penetration Disorder

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Genito-Pelvic Pain

Penetration Disorder

- Treatment
 - Cognitive Behavioral Therapy
 - Vaginal hormonal creams/vaginal lubricants
 - Treat any underlying infection/etiology
 - Pelvic floor/physical therapy
- Prognosis- variable, most often poor

- Treatment
 - Cognitive Behavioral Therapy
 - Benzodiazepine
 - Pelvic floor/physical therapy
 - OMM
- Prognosis – good with appropriate treatment

Male Erectile Disorder (ED)

Persistent or recurrent (at least one of following) :

- Difficulty obtaining erection

OR

- Difficulty maintaining erection

OR

- Decreased erectile rigidity

- **At least 6 months**

- **Distress**

- **Not** better fit for another **primary psychological condition**

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Male Erectile Disorder (ED)

- Epidemiology
 - < 40yo 1-9%
 - > 60yo 20-40%
- Differential Diagnosis
 - Age
 - Chronic medical condition
 - Hypertension, diabetes, other cardiovascular disease, hypogonadism
 - Psychological
 - Anxiety, depression, post-traumatic stress, prior poor experience
 - Medications
 - Antihypertensives, sedatives
 - Substance use
 - Smoking, alcohol, marijuana, cocaine
 - Surgery
 - Radiation therapy
- May lead to low self-esteem resulting in avoidance of sexual intercourse

Male Erectile Disorder (ED)

- Clinical Evaluation
 - Associated embarrassment, self-doubt, and loss of self confidence
- Testing
 - International Inventory of Erectile Function
 - Labs
- Treatment
 - Modification of risk factors/lifestyle
 - Phosphodiesterase type 5 (PDE-5) inhibitors
 - Cognitive behavioral therapy

Male Erectile Disorder (ED)

- Prognosis
 - Acquired psychogenic ED – excellent
 - Lifelong global ED – poor
 - Organic (mild/mod severity) – good with oral agents
 - Combination psychological/organic – varies (need psychotherapy and pharmacotherapy)

Delayed Ejaculation

In a male, persistent or recurrent (at least one) following normal sexual excitement phase :

- Delay in ejaculation

OR

- Infrequent ejaculation

OR

- Absence of ejaculation

- Lifelong or acquired

- **At least 6 months**

- **Distress**

- **Not better fit for another primary psychological condition**

Delayed Ejaculation

- Epidemiology
 - Exact prevalence is unknown
- Differential Diagnosis
 - Age
 - Medications
 - Antihypertensives, antidepressants/MAOI's/dopamine D-2 blockers, benzodiazepines/drugs causing alpha-adrenergic blockade)
 - Psychological
 - Depression
 - Chronic medical condition
 - Neurologic disorders, diabetes, hypertension, stroke, prostatitis
 - Substance useAlcohol, nicotine
 - Injury
 - Surgery

Delayed Ejaculation

- Testing
 - Pudendal nerve conduction study
- Treatment
 - Change medications (SSRI – side effect is delayed orgasm)
 - Education &/or psychosexual therapy
- Prognosis
 - Poor if psychological/medical in origin
 - Good if substance induced

Premature (Early) Ejaculation

In a male, persistent or recurrent ejaculation within 1 minute following:

- Vaginal penetration
AND
- Before individual (not partner) wishes it
- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**

Premature (Early) Ejaculation

- Epidemiology
 - Suspect ~1-3%
 - Scandinavian study
- Differential diagnose same as other ejaculatory disorders
- Etiology
 - Minimal evidence
 - Psychological factors (anxiety, panic d/o, PTSD, relationship)
 - Unconscious anger
 - Learned in adolescent → performance anxiety
 - Genetic

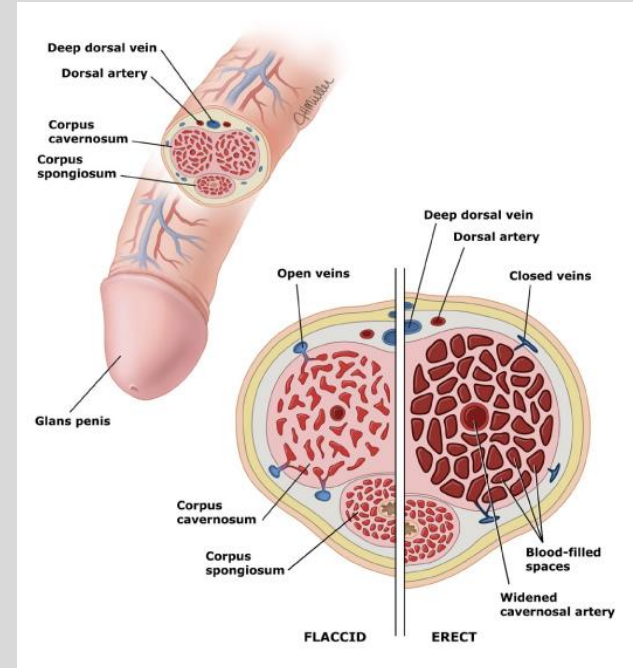
Premature (Early) Ejaculation

- Testing
 - Psychological testing not required for diagnosis
- Treatment
 - SSRI- paroxetine
 - Cognitive Behavioral Therapy
 - “Start-stop”
 - “Squeeze technique”
- Prognosis- good with treatment

Priapism

Persistent, usually painful erection lasting more than 4 hours in the absence of sexual excitation

- Epidemiology
 - 0.73 per 100,000 men per year
 - Bimodal peak distribution
 - 5 and 10 years in children
 - 20 to 50 years in adults
- Etiology
 - Pathologic engorgement of the corpora cavernosa but the glans penis and corpus spongiosum are not engorged and remain flaccid



Priapism

Veno-occlusive (ischemic)

- *Urological emergency*
- Reversible - sickle cell disease, direct injection of erectile agents, and leukemic infiltration
- Nonreversible – idiopathic (most common), spinal cord lesions, and medications

Arterial (non-ischemic)

- Self-limited
- Penile or perineal trauma

Priapism

- Clinical Evaluation
 - History/Physical Exam
 - Ischemic = typically painful, rigid erection
 - Non-ischemic = hx of trauma, less painful and rigid erection
 - Cavernosal blood gas analysis
 - Ischemic = black/hypoxemia/hypercarbia/acidemia
 - Non-ischemic = red/normal: oxygen/carbon dioxide/pH
 - Testing: CBC, Reticulocyte count, hemoglobin electrophoresis, peripheral blood smear, toxicology, Doppler ultrasonography

Priapism

- Treatment
 - Urology consult
 - Ischemic = Aspiration, with or without irrigation combined with an intracavernosal injection of a sympathomimetic drug (i.e. phenylephrine). If no response – shunt surgery
 - Non-ischemic = Observation, outpatient management
- Prognosis
 - 90% of men with an ischemic priapism lasting 24 hours do not regain the ability to have sexual intercourse
 - Early relief of priapism with return of normal flaccidity is not likely to be associated with long-term sequelae

Paraphilic Disorders

DSM-5

- Preference for sexual behaviors other than the forms of courtship and genital stimulation considered normal within a given society.

- Typically involve extreme, dangerous, and/or illegal behaviors
- Feel personal distress about their interest or have a desire that involves another person's psychological distress, injury or death.
- NO FDA approved treatment for paraphilias

Paraphilic Disorders

Paraphilia Behavior

- Preference for sexual behaviors (sexual fantasies or practices) outside of forms of courtship and genital stimulation considered normal within a given society

Paraphilic Disorder

- Paraphilic behavior with:
- Typically involve extreme, dangerous, and/or illegal sexual behaviors
- Feel personal distress about their interest or have a desire that involves another person's psychological distress, injury or death
- Rarely diagnosed
- NO FDA approved treatment for paraphilias
- DSM-5

Causes and Diagnosis

Unknown Causes

- Risk factors unknown
- More common in men
- Possible correlation to other behavior disorders

DSM 5

- Typically, more than fantasy; requires actions
 - Fantasy may cause distress, disabling, or depression
- Distress
- Present for at least 6 month
- May have more than one type

Paraphilic Disorder

Exhibitionistic Disorder

- Intense sexual arousal from EXPOSING of one's genitals to an unsuspecting individual(s)
- M > F (2-4% of people)
- Late teens / early adulthood
- Therapy (CBT) / medication
- High recidivism rate

Voyeuristic Disorder

- Intense sexual arousal from OBSERVING/VIEWING nudity of an unsuspecting individual(s)
- M > F
- Adolescent but best diagnosed >18 y/o
- Anti-depressant meds/possibly therapy
- Untreated has poor prognosis

Fetishistic Disorder

- Sexual arousal from objects or a specific body part which is not normally erotic
- Almost exclusively men
- Onset of puberty
- No specific treatment

Frotteuristic Disorder

- RUBBING OR TOUCHING a nonconsenting individual(s)
- Crowds
- Crime of opportunity
- Rarely preferred over other sexual activities
- M>F (30% of men)
- Peaks between 15-25 years of age with gradual decline in frequency
- Therapy (CBT) / medications (SSRI)

Transvestic Disorder

- Sexually aroused by the act of dressing in the clothes designated as the opposite gender manifesting as fantasies, urges, or behaviors
- 6 months / distress
- Childhood
- Almost exclusively male
- Therapy / medications (sertraline, lithium, estrogen, antiandrogens)

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[Scholar Rx Brick: Paraphilic Disorders](#)

Sexual Masochism Disorder

- Sexual gratification from RECEIVING abuse, humiliation, and domination
- M>F
- Sadism
- Early adulthood
- Possible treatment with therapy (CBT)

Sexual Sadism Disorder

- Sexual gratification from the psychological and physical suffering, humiliation of another
 - GIVING abuse, humiliation, domination
- Real / 6 months / distress
- M>F
- Early adulthood
- Therapy (CBT) / medications (SSRI, GnRH agonist)

Pedophilic Disorder

- Have sexual desire for prepubescent children
 - (fantasies, urges or behaviors)
- 16 years old or 5 years older than victim for diagnosis
 - Victim usually younger than 13 years old
- 6 months / distress
- Puberty
- Fondling / oral sex
- Behavior is the disorder
- Highly treatment resistant (therapy/meds) / high recidivism

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Summary

Now, the student should be on the way to:

- ✓ 1. Define the general terms of sexual dysfunction and paraphilic disorders.
- ✓ 2. Distinguishing specific sexual dysfunctions.
- ✓ 3. Applying diagnostic criteria to identify sexual dysfunctions.
- ✓ 4. Developing a clinical management plan for sexual dysfunctions and paraphilic disorders.
- ✓ 5. Recognizing specific paraphilic disorders.

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Resources and References

Scholar Rx Bricks:

Name of Discipline/ Organ System: Psychiatry

- **Title of Brick(s):** Sexual Dysfunctions

Link(s): <https://brick-portal.scholarrx.com/annotation/sexual-dysfunctions>

Name of Discipline/ Organ System: Psychiatry

- **Title of Brick(s):** Paraphilic Disorders

Link(s): <https://brick-portal.scholarrx.com/annotation/paraphilic-disorders>

Resources and References

Suggested/Supplemental Resources:

- [Psychosocial Issues and Female Sexuality](#). In: Hoffman BL, Schorge JO, Halvorson LM, Hamid CA, Corton MM, Schaffer JI. eds. *Williams Gynecology, 4e*. McGraw-Hill Education; 2020. Accessed February 16, 2024.
- [Sexual Dysfunction and Paraphilias](#). In: Balon R, Segraves R (2019). Chapter 24. In Ebert M.H., Leckman J.F., Petrakis I.L. (Eds), *Current Diagnosis & Treatment: Psychiatry, 3e* New York, NY: McGraw-Hill.

Student Dedicated Hours

In-Person or Virtual Office Hours

Every Friday 11a-12p CST / 12-1p EST

→ Office 424D

→ Via Zoom: <https://nyit.zoom.us/my/aloftisdo> (Join: aloftisdo)

Via Appointment

→ Through email: aloftis@nyit.edu

→ By phone: 870.680.8817

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